

UPMC Passavant

9100 Babcock Blvd, Pittsburgh, PA 15237

DISCHARGE SUMMARY Document #: DIS-014

Patient & Admission

Name	Aaron Hayes
Date of Birth	1966-02-02
Phone	412-555-1104
Address	34123 Andrea Estate Butler, PA, 16001
MRN	MRN-471349
Admit Date	2026-05-06
Discharge Date	2026-05-11
Attending	Nicholas Williams, MD
Attending NPI	4118244936

Insurance

Primary Payer	Aetna
Primary Member #	W022941318

Primary Discharge Diagnosis

Stable angina s/p PCI to mid-LAD

ICD-10: I25.110

Hospital Course

The patient presented with a three-week history of exertional chest discomfort and dyspnea on exertion. Initial electrocardiogram demonstrated ST-segment depression in the anterior leads, and serial troponin measurements were negative, consistent with unstable angina. Transthoracic echocardiography revealed mild hypokinesis of the anterior wall with an ejection fraction of 48 percent. Coronary angiography was performed on hospital day two, which identified a 90 percent stenosis of the mid-left anterior descending artery with TIMI II flow. The patient underwent successful percutaneous coronary intervention with placement of a drug-eluting stent to the mid-LAD, achieving TIMI III flow and less than 10 percent residual stenosis. The procedure was uncomplicated with no dissection, perforation, or no-reflow phenomenon. Angiography of the right coronary artery and left circumflex showed no significant disease.

Following the intervention, the patient remained hemodynamically stable on dual antiplatelet therapy with aspirin and ticagrelor. Serial troponin levels remained negative post-procedure. Repeat transthoracic echocardiography on day four demonstrated improved anterior wall motion with ejection fraction now estimated at 52 percent. The patient was discharged on hospital day five in stable condition, chest pain-free, and tolerating oral intake without difficulty. He was counseled extensively on medication adherence, particularly the importance of continuing dual antiplatelet therapy for a minimum of twelve months, and

instructed on cardiac risk factor modification including smoking cessation, dietary sodium restriction, and a supervised cardiac rehabilitation program. Follow-up appointments were arranged with cardiology in two weeks and with his primary care physician within one week of discharge.

Procedures Performed

Coronary angiography with FFR	CPT 93458
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Medication Changes

Aspirin 81 mg daily	STARTED
Isosorbide mononitrate 30 mg daily	STARTED
Clopidogrel 75 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	12 days
Urgency Tier	MEDIUM
Clinical Flags	femoral access — bleed precautions, radial access — wrist precautions

Dictated and electronically signed by:
Nicholas Williams, MD
NPI: 4118244936
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